SITUATION REPORT C0VID -19 | ZIMBABWE			
Date of Issue	07 April 2020	**Investigation Start Date**	25 January 2020
Data as Reported by	06 April 2020	**Prepared By**	MoHCC

1 GLOBAL UPDATES SNAPSHORT:

- As of 06 April 2020, a cumulative of 1 210 956 confirmed cases and 67 594 deaths with a CFR of 5.5 % were reported from more than 203 countries/territories. (Source WHO Sitrep No.77).

2. ZIMBABWE – HIGHLIGHTS

- Today the 7th of April 2020 The National Microbiology Reference Laboratory tested 21 samples and 1 tested positive for COVID-19.
- Zimbabwe has reported 11 confirmed cases with two deaths since the onset of the outbreak.
- The first imported case was reported on the 21st of March 2020.
- Local transmission started on the 24th of March 2020.
- Ministry of Health and Child Care COVID -19 Call Centre hotlines (2019) is now in use 24/7.

ZIMBABWE –COVID19 CONFIRMED POSITIVE CASES

PROVINCE	NO OF CONFIRMED CASES
Matabeleland North	1
Bulawayo	1
Harare	5
Mashonaland East	4
Total	**11**

3.1 Coordination

Following the declaration of COVID-19 as a national disaster by HE President E.D Mnangagwa on the 19th of March 2020; the following coordination activities have been conducted;

- High level political commitment to the National COVID-19 response with appointment of Honourable VP KCB Mohadi as the Chairperson of the Ad hoc Inter-ministerial taskforce on COVID- 19 which continues to meet on a regular basis.
- Creation of eight sub-committees of the Inter-ministerial Task force on COVID-19
- Declaration of a 21-day nation- wide lockdown except for essential services started on the 30th March 2020 and is being largely adhered to except for a few places whereby enforcements have been deemed necessary.

Table 1: Summary COVID-19 Response Pillars

Overall high-level technical planning and coordination of the National Preparedness and Response Plan is led by the Permanent Secretary for Health and Child Care Dr. A. Mahomva working with permanent secretaries of other ministries in support of the Inter-ministerial COVID-19 Task force.

Pillar	Pillar Lead	Pillar Lead & Contact Details	Scheduled Meeting
1.Country-level coordination, planning and monitoring	Dr Mhlanga Dr P. Manangazira	0717 862 600 0773 405 630	Daily
2.Points of entry	Mr. Nyamandi	0772 809 365	Daily
3.National Laboratory	Simbi/Mazarura	0772 213 357/	Daily
4.Case Management	Mr. Makarawo Dr Chifamba	0773 288 111	Daily
5.Infection Prevention and control	Dr Chasokela Professor V Robertson	0712 878 014	Daily
6.Surveillance, Rapid response teams and case investigation	Dr I Phiri	0772 810 580	Daily
7.Risk communication, and community engagement	Mr. Chinakidzwa	0772 737 046	Daily
8.Operational support and logistics	Mr. Mudzimu	0772 969 825	Daily
9.Law enforcement and Security	Dr. Fusire	0772 225 700	

Note:

Each pillar is expected to submit daily updates to the COVID-19 Coordinators Dr. Mhlanga and Dr. Manangazira by 12:00pm for onward submission to and discussion with the Permanent Secretary for Health and Child Care every morning.

Weekly High Level Technical coordination meetings chaired by the Permanent Secretary for Health and Child Care are scheduled to be held every Tuesday morning in the Emergency operations Centre situated at Parirenyatwa Hospital Grounds.

- The high-level Technical coordination meeting, chaired by MOHCC, involving Permanent Secretaries from all Ministries or their representatives, UN partners and donors was held on Tuesday 31 March 2020 and 07 April 2020.
- **COVID-19 Response Plan**: Application for prioritized action plan for COVID-19 response has been developed and submitted to partners.
- **Coordination:** Preparedness activities have continued at national and subnational levels since the activation of these structures in January 2020. The RRTs meet on a regular basis to monitor implementation, preparedness and response activities including surveillance of travelers from COVID-19 affected countries.
- **Partner coordination:** The Minister and Permanent Secretary for Health and Child Care continued to engage with key stakeholders to discuss on funding and implementation for the COVID 19 preparedness and response activities in line with the 9 pillars of response
- **Civil Protection:** A meeting of the national civil protection committee committed to develop TORs for the subcommittees, and a meeting with combined security forces made recommendations to the Chief Director Preventive Services and offered to support the MOHCC in the COVID-19 response.
- **ZRP offered 10 water canon vehicles** each with a capacity of 6 000 liters for chemical disinfection of identified hot spots, (public spaces, bus termini, major markets) to enable coverage of all major cities, provinces and ports.

3.2 Points of Entry and Surveillance

Screening of passengers arriving from COVID-19 affected countries commenced on the 22nd January 2020.

As of 6 April 2020, 20 512 passengers arriving from countries with confirmed COVID-19 cases were screened at various ports of entry.

Table 2. Number of travelers arriving by port of entry from all countries reporting cases as at 30 March 2020.

Point of Entry	Arrivals put on Surveillance	Arrivals removed from Surveillance	Arrivals on Surveillance
RGM Int Airport	14077	9218	4859
JMK Int Airport	1871	1799	72
Victoria Fall border	1053	801	252
Victoria Fall Airport	1373	1350	23
Beit bridge	756	515	241
Mangwe/Plumtree	434	164	270
Forbes	44	0	44
Nyamapanda	1	1	1
Kazungula	903	871	32
Total	**20512**	**14719**	**5793**

- On the 1st of April 2020 the Surveillance pillar held a virtual training for the RRT for Hwange District on the following: surveillance tools investigation form, contact line listing form, daily contact monitoring form and contact summary form.
- Surveillance pillar developed case investigation and other surveillance tools for contact tracing and these have been shared with the provinces.

Table 3 Contact tracing for COVID-19 confirmed cases

Cases	Region	Total Number of Contact Listed	Total Number of Contacts Followed up	New Contacts followed Up	Total Number of Contacts reporting Symptoms	Total Number Symptomatic suspects referred for further investigation	Total Number of Contacts Finished	Total Number of Contacts dropped	Imported/Local transmission
Case 001	Matabeleland North	12	12	0	3	3	0	0	Imported
Case 002	Harare	37	37	0	4	4	9	1	Imported
Case 003	Harare	24	24	4	2	2	0	2	Imported
Case 004	Harare	3	3	0	0	0	0	0	Imported
Case 005	Harare	3	3	0	0	0	0	0	Local transmission
Case 006	Harare	2	2	0	0	0	0	0	Local transmission
Case 007	Harare	14	14	0	1	1	0	0	Local transmission
Case 008	Harare	5	5	5	-	-	-	-	Local transmission
Case 009	Harare	5	5	5	-	-	-	-	Imported
Case 10	Harare	5	5	5	-	-	-	-	Imported
Case 11	Bulawayo	-	-	-	-	-	-		Local transmission
Total		110	108	19	10	10	9	3	

3.4 Laboratory

The National Microbiology Reference Laboratory continued to provide COVID-19 testing services as detailed below

Table 5 NMRL Laboratory Summary Statistics: 04 April 2020

Province	No of Specimens received last 24 hours	Number of Tests Done	Positives	Negatives	Indeterminate
Harare	20	21	0	20	0
Mat. North	0	0	0	0	0
Mat. South	0	0	0	0	0
Midlands	0	0	0	0	0
Mash East	0	0	0	0	0
Mash West	0	0	0	0	0
Mash Central	0	0	0	0	0
Manicaland	0	0	0	0	0
Masvingo	0	0	0	0	0
Bulawayo	1	0	1	0	0
Total	**21**	**21**	**1**	**20**	**0**

NB The break down by provinces not updated the information not available at the time of print.

Cumulative tests (National)

Total Tested	395
Total positive	11
Total Negative	384

Stock Status of PCR reagents

Reagent	Number of tests remaining
Extraction Reagent	85
Amplification Reagent	19774

Stock Status for PPE and related commodities

PPE and Collection Swabs

ITEM	Quantity in Stock
Labcoats	60
Face Masks	600
Head Gear	0
Shoe Covers	0
Gloves	10 boxes
Sample collection swabs (with media)	100
Goggles	60
Face shields	0

Rapid Diagnostic test kits

- 2000 COVID-19 Rapid diagnostic tests procured by Ecobank were received by the NMRL.
- These are currently under evaluation and validation.

3.5 Case Management and Infection Prevention and Control (IPC)

Key activities

- Pillar 6 Meeting was held on Monday 6 April 2020 via ZOOM supported by UNICEF
- COVID-19 PPEs Policy completed and this was circulated to PS, & others last week.
- Provincial and hospital IPC Focal points were identified and training was conducted last Thursday & Friday.
- The committee submitted training budget to the Chief Director – Curative Services for GOZ Funding.
- Patient referral pathways to be finalized with the Case Management Pillar protocol
- 7 health facilities assessed in Harare for use as COVID-19 isolation centre. A similar exercise is ongoing in Bulawayo and 8 provincial centres.
- Case Management protocols, standards and regulatory frameworks were reviewed.
- An assessment of diagnostic, therapeutic and vaccines for compassionate use was completed and the report will be shared widely in due course.

3.7 Risk Communication and Community Engagement

The National Risk Communication Committee Meeting

- Met on 31 March 2020 and deliberated on rolling out interventions in the provinces
- Partners asked to complete 4W matrix and submit to Coordination Chairs. MoHCC and pledge funding for the implementation of the activities.
- Committee is conducting a virtual meeting bi-weekly.

- Committee being co-chaired by Ministry of Information, Media and broadcasting services and MOHCC.

Adverts on Radio

- MoHCC is collaborating with Plan International in dissemination of information through community radio stations in the provinces
- Audios on COVID-19 have started airing on the 2nd April 2020 after the approval of the audios by Chief Director Preventive Services.
- Bulawayo City have also developed audios in Ndebele and are airing on Khulumani radio station
- Provinces are making use of community radio stations such as Hevoi, Ya FM, 98.4FM, Diamond FM, Khulumani FM to disseminate information.
- The National AIDS Council has bought airtime at Ya FM and 98.4 which health staff in the Midlands is using.

IEC Materials

- All major shops in the cities have posters on COVID-19.
- Advisories on COVID-19 treatment, social distancing at food outlets, use of masks and what to do if one suspects COVID 19 at home have been handed over to Ministry of Information for wider circulation.

Activities in the Provinces

- Provinces submit their risk communication activity reports bi-weekly to the national level
- Food distribution points are being used in the provinces as points of sharing information on COVID-19. An HPO or EHT are responsible for disseminating information to these points
- Harare, Gweru, Bulawayo and Chinhoyi are disseminating information to the public using loud hailers
- Local leadership in Shamva played in active role in enforcing compliance in the lockdown

3.8 Logistics and operational support.

- A supply of PPES and test kits has been received and distribution of supplies is currently ongoing.
- The Government of Zimbabwe procured of 18 000 litres of fuel and airtime for communication and coordination. This has been distrusted to all Provinces, Cities and the NRRT. There is ongoing procurement and distribution of PPE and general commodities.
- Central Hospitals and institutions under the Ministry of Higher and Tertiary Education are manufacturing alcohol-based hand-rubs, staff uniforms and other PPES to strengthen hand washing and IPC.

4. PRIORITY ACTIONS

- Decentralization of COVID-19 PCR testing to provincial and district level.
- To increase testing of more suspected cases of COVID-19 to at least 33000 per month. This will be done through increasing testing in- patients with pneumonia and out-patients with flu like illnesses.
- Strengthening contact tracing for the confirmed cases in view of local transmission
- Strengthening coordination for COVID 19 across all the 9 pillars.
- Implementation of the recently launched COVID-19 preparedness and response plan
- Strengthen capacity for entry screening of all arrivals at the international airports and key ground ports of entry following confirmation of imported cases.
- Intensify in-country surveillance in view of border closures, deportations and migration from urban to rural areas,
- Ensure daily submission of all screening data for analysis, and reporting
- Scale up training of key health workers at identified isolation sites, in conformity with WHO guidelines for detection and response to COVID-19 transmission.
- Finalize ongoing activities aimed at strengthening in-country capacity for laboratory diagnosis of COVID-19.
- Daily meetings and reporting of all key national technical and teams overseeing the implementation of priority response activities according to the COVID-19 Plan Pillars.
- Develop/ strengthen institutional arrangements with private health care actors to coordinate information on screening, testing and treatment of COVID-19 as well as indications of continuing operations and health service delivery.

Annex 1 COVID-19 Case Definitions

COVID-19 Case Definition

Case definitions for surveillance

The case definitions are based on the current information available and will be revised as new information accumulates. Countries may need to adapt case definitions depending on their own epidemiological situation.

Suspect case

A. A patient with acute respiratory illness (fever and at least one sign/symptom of respiratory disease (e.g., cough, shortness of breath), AND with no other etiology that fully explains the clinical presentation AND a history of travel to or residence in a country/area or territory reporting local transmission of COVID-19 disease during the 14 days prior to symptom onset.

OR

B. A patient with any acute respiratory illness AND having been in contact with a confirmed or probable COVID-19 case (see definition of contact) in the last 14 days prior to onset of symptoms;

OR

C. A patient with severe acute respiratory infection (fever and at least one sign/symptom of respiratory disease (e.g., cough, shortness breath) AND requiring hospitalization AND with no other etiology that fully explains the clinical presentation.

Probable case

A suspect case for whom testing for COVID-19 is inconclusive

Confirmed case

A person with laboratory confirmation of COVID-19 infection, irrespective of clinical signs and symptoms.

Recovered Case See definition in the next issues

Definition of contact

A contact is a person who experienced any one of the following exposures during the 2 days before and the 14 days after the onset of symptoms of a probable or confirmed case:

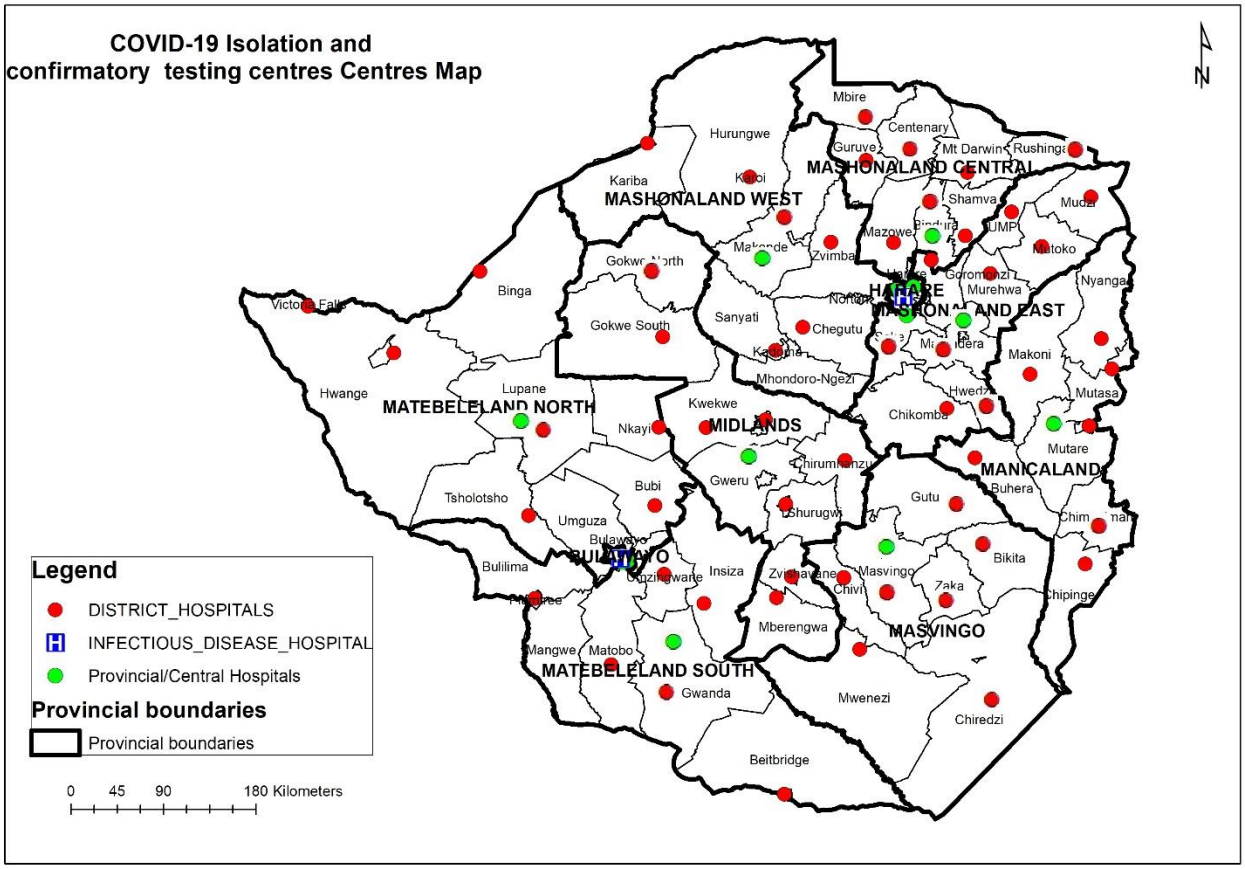
1. Face-to-face contact with a probable or confirmed case within 1 meter and for more than 15 minutes;

2. Direct physical contact with a probable or confirmed case;

3. Direct care for a patient with probable or confirmed COVID-19 disease without using proper personal protective equipment; OR

4. Other situations as indicated by local risk assessments

Annex 2: Map showing location of COVID-19 Isolation and Treatment Centers



World Health Organization

This SitRep was compiled by the MOHCC Health Information and Surveillance Unit, Epidemiology and Disease Control Directorate in collaboration with WHO Country Office.